

Class Topic: Just Distribution of Scarce Medical Resources

Part 1: Video Outlines (3 points)

Video 1: The Ethics of Medicine: A Third Heart Transplant

I. AYANA LUCAS

- A. Received her third heart transplant
- B. She is 8 years old.
- C. Children donors are rare.
- D. Her first heart failed.
 - 1. She went to the top of the UNOS list.
- E. Her second heart failed.
 - 1. She received her third heart as the sickest patient.

II. THIRD AND FOURTH HEART TRANSPLANTS STATISTICS

- A. The body builds antibodies with each new heart.
- B. Third heart transplants 1 year survival statistics drop to 10%
- C. Almost 0% 1 year survival for fourth heart.

III. THE RIGHT USE OF HEARTS

- A. Should first time transplants with better statistics get priority over Ayana Lucas?
- B. Should the system limit the maximum number of transplants?
 - 1. The speaker believes that it should.
 - 2. Doctors would make an emotional decision bedside.
- C. A well-informed system should control limited resource allocation.

Video 2: UNOS Allocation and Deceased Donor Criteria

I. UNOS BACKGROUND

- A. Unified Network for Organ Sharing
- B. Non-profit, charitable membership organization
- C. Maintains national transplant waiting list
- D. Establishes allocation policies.
- E. Educates & monitors members for complaints
- F. Collects & analyzes data

II. ORGAN ALLOCATION

- A. Balances justice and utility
- B. Consider:
 - 1. Waiting time
 - 2. Medical urgency
 - 3. Net benefit
- C. Donations are donor-driven.
 - 1. A new list is generated for each donor.
 - 2. A computer matching system matches donors to organs.
- D. Allocation is local, not regional.

III. KIDNEY ALLOCATION

- A. No medical eligibility requirements.
- B. Waiting time:
 - 1. Starts when registered under 18-years-old.
 - 2. Starts at dialysis date when over 18.
- C. Kidney Donor Profile Index (KDPI)

1. Estimate kidney quality
2. Considers and weights variables
3. High-KDPI kidneys could be usable

D. Estimated Post-Transplant Survival (EPTS)

1. Score that measures the likelihood of patient survival
2. Drives how patients come up the transplant list

E. Patients are divided into sequences A through D

1. Lower sequences:
 - a. are higher priority.
 - b. require kidneys with lower KDP.

F. Software can match kidneys to recipients.

IV. KIDNEY/PANCREAS LISTING CRITERIA

A. Recipients must be:

1. Registered on the kidney waitlist.
2. Diagnosed with diabetes or pancreatic exocrine insufficiency.

B. Waiting time

1. Begins when registered for younger than 18.
2. Begins when the patient requires insulin.

C. In reality, the kidney and pancreas lists are one list

1. Sequence one: donor age 50 or less and with BMI 30 or less.
2. Sequence two: donor over aged over 50 or with BMI greater than 30.

I. ARTIFICIAL KIDNEYS

A. These machines:

1. Filter blood through cellophane.
2. Washed in a solution called dialysate.

B. The patients attached to artificial kidneys cannot move.

II. SEATTLE ARTIFICIAL KIDNEY CENTER SCREENING

A. Medical committee:

1. Decides if the applicant is medically fit.

B. Finance committee:

1. Discovers if the patient can pay:
 - a) \$10,000 per year.
 - b) The first three years upfront.
2. Helps the patient raise the money if they can.

C. Admissions Advisory Committee:

1. The applicant must be approved by a group of volunteers.
2. They decide if the patient is “worth” saving.

III. THE ADVISORY COMMITTEE

A. The members are anonymous.

B. The applicants are anonymous to the committee.

C. Buddy Franklin case:

1. Kidney disease
2. His town helped raise the funds
3. The committee valued his community impact

4. He was approved.

D. Phylis Miller case

1. 28-year-old mother of three
2. Complete kidney failure

E. Norman Hozier case

1. 43-year-old father of three
2. Rejects help for funding
3. Believes the government should pay because he got sick in war
4. He withdrew his application

IV. DR. WILLEM KLOFF

- A. He was from Holland.
- B. Created the first dialysis machine.
- C. First patient treated in 1943.
 1. They died.
- D. The 17th patient survived.
- E. The original machine required a new operation every session.
 1. His version was not practical.

V. DR. BELDING SCRIBNER

- A. Made a device to treat chronic kidney disease.
- B. Dialysis would no longer require a new operation each time.
 1. Physicians could just remove the stunt and connect to the artificial kidney.
- C. These machines are limitedly accessible due to the high price.

VI. THE FIGHT FOR FUNDING:

- A. Doctors spoke with state representatives directly.
 - 1. Dr. Morton H. Maxwell
 - 2. Dr. Edward H. Kass
 - 3. Dr. Theodore Tsaltas
- B. Proposed a program that would eventually cost \$400 million per year.
 - 1. The funding would come from the federal government.
 - 2. It places a value on human life.
- C. The doctors call for an increased focus on human issues.
- D. The American Medical Association has not taken a stance on the artificial kidney.
- E. The artificial kidney works without doubt.
 - 1. The issue is funding.

VII. HOME DIALYSIS:

- A. Could save costs of nurses and doctors.
- B. Allows for a greater sense of normalcy.
- C. Requires a two-month training period before home implementation.
 - 1. No experience required.

Part 2: Chapter Information (3 points)

- Just distribution of scarce medical resources and personal responsibility (Pence, 2021, p. 266).
- Willem Kloff invented the dialysis machine and Scribner invented the first permanent stunt; the life saving invention was limited (Pence, 2021, pp. 266-267).
- King County Medical Society created a committee to decide who would get dialysis machines, using strict physical and personal guidelines (Pence, 2021, p. 267).
- The God Committee had stories in major publications (Pence, 2021, p. 268).
- End Stage Renal Disease Act (ESRDA) mandated the government to pay for dialysis machines (Pence, 2021, p. 269).
- Scribner's public moral dilemma led to a rise in public bioethics discussion (Pence, 2021, p. 270).
- Mandated choice requires adults with a driver's license to indicate their choice of being an organ donor; required request mandates that the hospital asks a patient's relatives (Pence, 2021, p. 270).
- Some European countries use presumed consent, where everyone who has not declined is presumed a donor (Pence, 2021, p. 271).
- Ethical issues in allocating scarce medical resources (Pence, 2021, p. 271):
 - The God Committee valued social worth.
 - Ernie Crowfeather received dialysis from the God Committee, but he continued to drink, turn down therapies, and eventually died (Pence, 2021, p. 271).
 - There is a debate surrounding a person's responsibility to their health and if that should impact their chance to get the scarce resource (Pence, 2021, p. 272).

- Kantian ethics holds that alcoholics should be held responsible but also that a scarce resource should be fairly decided through random draw (Pence, 2021, p. 273).
- Celebrities and wealthy people have greater access to organs (Pence, 2021, pp. 274-275).
- Retransplants involve giving a needy patient another after their body rejected the first (Pence, 2021, p. 276)
- Albert Jonsen's rule of rescue involves giving scarce resources to named patients; many issues surround the value of unidentified versus known patients (Pence, 2021, p. 277).
- Congress mandated that UNOS allocates to the sickest first (Pence, 2021, p. 279).
- There are issues surrounding living donors, as the donor is harmed to benefit another person and many donors require transplants later (Pence, 2021, p. 279).
- Non-Heart-Beating Cadaver Donors (NHBCDs) follow Do Not Resuscitate (DNR) orders, which ignited conversation about hastening deaths for patients (Pence, 2021, pp. 282-284).
- Many critics of the God Committee also struggled to allocate the resources well, as dialysis is a difficult process (Pence, 2021, pp. 284-285).
- Expensive Nocturnal or Rogosin home dialysis alleviated patients' need to go to the office for treatment, for a significant price (Pence, 2021, p. 286).

References

Pence, G. E. (2021). *Medical Ethics: Accounts of Ground-breaking Cases*. McGraw-Hill.

Part 3: Two Discussion Questions (3 points)

1. In getting a transplanted organ that saves your life, which of the following should a just system of allocation consider?

Whether the patient smoked; whether the patient drank alcohol extensively; whether the patient has medical insurance; whether the patient is rich and is listed in several medical centers; whether the patient has already received one organ transplant.

I do not believe that the listed qualities should be significant in a just allocation system. To me, a just system would distribute organs to maximize the number of people who survive, preferably for a long time. To support this, it would consider whether the patient smoked, drank extensively, and had already received one organ. Knowing if the patient smoked or drank alcohol extensively may be important, as these factors could impact the immediate and long-term success of the procedure. This information would help ensure that the organ transplant is successful long-term, but I do not believe it should be the basis for rejecting a patient without further reason. The book mentions that, “82 percent of first-time transplants survive one year, but only 57 percent of retransplants do” (Pence, 2021, p. 276). This information should not immediately lead to the rejection of these patients, but it should be considered when evaluating the case. The proposed just system should disregard a patient's medical insurance, wealth, and celebrity status, as these factors would introduce unfair bias.

2. Isn't it human nature to rescue the sickest first? To stave off death from someone you know? Do we want surgeons to be bureaucratic robots or to have a heart? What's wrong with the rule of rescue and caring for patients who are known?

I do not believe that it is human nature to rescue the sickest first. Utilitarians may let the sickest person die to maximize the number of patients who live or the number of years per organ. There are other instinctive perspectives. I agree that it is human nature to stave off death from someone you know, as emotions would propel most to make that decision. Surgeons should have fair, well-informed guidelines to follow in difficult decisions, but the lack of one makes me prefer that the ideal surgeon should have a heart. I feel that their burden of ethical decisions should be minimized. The major problem with the rule of rescue and caring for known patients is that it implies that the well-known donor is more valuable. It creates an emotional bias that disrupts fairness, an objective view of the situation, and established procedures by favoring individuals.

References

Pence, G. E. (2021). *Medical Ethics: Accounts of Ground-breaking Cases*. McGraw-Hill.